DPP	DEPARTMENTAL POLICY AND PROCEDURE		
Title	HANDOVER OF PATIENT FROM NICU TO WARD		
Department	Neonatology Department		
Applies to	All NICU Medical and Nursing Staff		
Policy Code and Version Number	DPP-NICU-019 (v1)		
Last Version Approval Date	N/A	Number of Pages	4

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1.0 PURPOSE

1.1. To provide guidelines on how to handover patient from NICII to ward.

1.2. To ensure the continuity of care after receiving the patient in the ward.

2.0 DEFINITION/ABBREVIATION

2.1. **Computed Tomography scan (CT)** - uses special X-rays to make detailed images of the structure inside the body.

2.2. **Magnetic Resonance Imaging (MRI)** - uses powerful magnetic fields and radio frequency to produce detailed pictures of body structures but is often clearer than CT scan images.

2.3. **New born Screening (NBS)** - test of new born to screen for serious treatable metabolic disorders.

2.4. **NOD**- Nurse on Duty

2.5. **HN**- Head Nurse

2.6. **CN**- Charge Nurse

2.7. **P.R.O**.- Patient Relation Officer

2.8. **NICU**- Neonatal Intensive Care Unit

3.0 RESPONSIBILITY

3.1. **Head of NICU**

3.1.1. Responsible for ensuring that patient meets the criteria of discharge/transfer before transporting the patient to ward.

3.2. **NICU Physicians (Residents, Specialist, Consultant)**

3.2.1. Provide a written order for transfer of patient to ward.

3.3. **NICU Nurse (Head nurse, Charge Nurse, Staff nurse)**

3.3.1. Confirms the availability of bed in the ward prior to shifting the patient.

3.3.2. Inform the Patient-Relation Officer to ask the parents to come to NICU to allow the mother to stay with the patient in the ward.

3.3.3. The nurse is responsible to transfer the patient from NICU to ward with proper endorsement given to the HN/CN and or receiving unit nurse

3.4. **Ward Nurse**

3.4.1. Prepares the neonatal crib and the machines needed prior to receiving the patient in the unit

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4.0 POLICY

4.1. Transfer of patient to the ward will be based on the NICU Discharge criteria and Physician's order.

4.2. The NICU physician and nurse must completely write the NICU Internal transfer prior to shifting the patient to ward.

4.3. The transferring and receiving nurse and physician must both sign in the NICU Internal Transfer form after endorsement that will serve as evidence for handover care.

4.4. The NICU nurse will give complete endorsement to the ward nurse with emphasis on:

4.4.1. Patient's complete data

4.4.2. Patient's admitting diagnosis

4.4.3. Latest vital signs

4.4.4. Current medications

4.4.5. Medications previously received

4.4.6. Laboratory investigations sent and to be done

4.4.7. Any diagnostic procedure done (Ultrasound, X-ray, CT, MRI) etc. 4.4.8. Any consultations done (Ophthalmology, Dermatology) etc.

4.4.8. Any vaccinations received

4.4.9. Any screening done and reports (NBS, Hearing Screening test, Cardiac Screening test)

4.4.10. Birth certificate (if already handed down to parents)

4.5. Parents will be notified about the transfer of patient to ward through designated office (PRO).

5.0 PROCEDURE (S)

5.1. The Head of NICU along with the NICU physicians will assess the patient if qualifies for Discharge criteria to facilitate transfer of patient to the ward if cannot be directly discharge from NICU.

5.2. The NICU Physician on-call will write the order of transfer in the order sheet and will fill up the NICU internal transfer form.

5.3. The NICU nurse will confirm first by calling the receiving ward nurse for the availability of bed in the ward.

5.4. If bed is available, then the NICU nurse will call the PRO to inform the parents to come to NICU to start the transfer procedure.

5.5. Inform the NOD in the ward about the patient's present condition, take the possible room number and verify with the ward HN or CN about the time of transfer.

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5.6. Once the parents arrived in NICU, confirm their identity and advise them to go to the ward to allow the mother to stay in the specified room number.

5.7. Check the completeness of the patient's medical file.

5.8. Prepare the patient and the machines needed for transport if necessary.

5.9. The assigned NICU nurse will transport the patient to the specified ward via open crib or transport incubator if needed.

5.10. The NICU nurse will give complete and proper endorsement to the ward nurse according to policy.

5.11. The NICU nurse should re-emphasize the health teachings given to parents which starts from admission time. (

5.12. After transfer of patient, the NICU nurse will clean the used incubator and machines according to infection control policy as soon as possible for the next admission.

5.13. Discard all materials used by the patient according to policy.

5.14. Record the patient's transfer data in the Bed Management Logbook and submit to nursing supervisor.

5.15. Transfer patient's bed account to the ward through Bed Management System.

5.16. Record all the relevant information of patient transfer in the NICU Admission/Discharge logbook.

6.0 EQUIPMENTS

6.1. Crib

7.0 ATTACHMENT/FORMS

7.1. Nurses Progress Notes (GDOH-NUR-NPN-203)
7.2. Physician Order Form (GDOH-INP-POF-079)
7.3. Handover Form (APP-QPS-HOF-007)

8.0 REFERENCES

8.1. MOH Guidelines for Neonatal Care

8.2. DPP-NICU-0002(v2): NICU DISCHARGE CRITERIA

8.3. Previous PMSH policies and procedure,20lT revision

9.0 REVISION CLAUSE

9.1. Not Applicable

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KINGDOM OF SAUDI ARABIA

وزارة الصحة
Ministry of Health

Hospital: _____ مستشفى:

Region: _____ المنطقة/المحافظة:

Dept./Unit: _____ القسم/الوحدة:

MRN: | | | | | | | | | | | | رقم الملف الطبي:

Name: _____ الاسم:

Nationality: _____ الجنسية:

Age: _____ ☐ سنة Years ☐ شهر Months ☐ يوم Days العمر:

Date of Birth: ___ / ___ / 14___ H ___ / ___ / 20___ تاريخ الميلاد:

Gender: ☐ Male ☐ Female الجنس:

NURSES PROGRESS NOTES FORM

DATE &TIME	NURSES NOTES (D-data A-action R-response)	REGISTERED NURSE INITIAL WITH ID NUMBER

Note: Write the time in each entry & affix your initial at the end of each paragraph. Document your complete Name, Initial, Job number, Date & Time at the closure of your documentation. Draw a line across empty spaces.

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Name: _____ الاسم:

Nationality: _____ الجنسية:

Age: _____ ☐ سنة Years ☐ شهر Months ☐ يوم Days العمر:

Date of Birth: ____ / ____ / 14____ H ____ / ____ / 20____ تاريخ الميلاد:

Gender: ☐ Male ☐ Female الجنس:

PHYSICIAN ORDER FORM

ALLERGIC TO:

DATE	TIME	PHYSICIAN ORDERS	PHYSICIAN STAMP / SIGNATURE	RN INITIAL / JOB NUMBER

MIKHWA GENERAL HOSPITAL
AL-MIKHWA, AL- BAHA, KSA

MRN

Name: _____

Age: ☐☐ Yrs/Months Sex: M ☐ F ☐

Nationality: _____

Consultant In charge: _____

Dept: _____ Unit: _____

HANDOVER FORM

DATE: **TIME:**

Situation

Outgoing Staff Nurse: _____ **Signature:** _____ **Job Number:** _____	**Incoming Staff Nurse:** _____ **Signature:** _____ **Job Number:** _____

Admitting Physician:	**Admission Date:**	**Diagnosis:**	
Admitted From:	**Diet:**	**Allergies:**	
Infections:		**Next Lab Due:**	**HAI: Yes __ No __**

Background

History of Present Complaints: **Prior medical & social History:**	**Pending Medications:**
Fall assessment done? Yes __ No __ **Indicate Risk:**	**Pressure Ulcer Risk Assessment Done? Yes __ No__** **Indicate Risk Level:**
Skin Assessment Done? Yes __ No__ **Wound Care: ___ hourly , Dressing type: _____**	**Restrain order valid: Yes __ No__ Due Date:**

Assessment

T: ___ °C BP: ___ / ___ mmHg HR: ___ / min **RR: ___ / min SPO2: ___ % GCS: ___ / 15**	**Code Status: ___ Full Code ___ DNR**	
Pain Scale: ___ / 10 Pain Management:	**Intake:**	**Output: ___ml / 8 / hours**

Respiratory: On Oxygen Therapy? Yes __ No__ / O2__ LPM via NC__ Mask__ NRFM__ TRACHY __ ATC__ ETT__

Nursing Care Plan Goals Evaluated: Yes __ No__ New Problems Identified:

IV Cannula? Yes __ No___ Site: _____ Gauge: _____ Date Inserted: _____

IV fluid: Yes __ No___ Type: _____ Rate: _____ml/hr. Date Commenced: _____

Recommendation

Blood Transfusion? Yes ___ No ___	**Date**	**Time**	**Comments:**

Pending Labs: Done: Yes ___ No___ **Result:**

Urgent Abnormal Labs: Yes ___ No___ See Attached Printout:

	Done:	Results:	Comments/ Notes:
X-ray:	**Yes ___ No___**		
Procedures:	**Yes ___ No___**		
Special Medications	**Yes ___ No___**		
Consultations:	**Yes ___ No___**		

NAME:	MRN:

#	CRITERIA TO COMPLETE:	
1	**ID BAND**	
	• Correct ID band and readable	
2	**IV SITES & IV LINE**	
	• Date of insertion • Condition of IV site	• Nurse's Name, Signature &Number • IV line, IV fluid labeled with date, time, signature of nurse
3	**WOUND DRAINS AND CATHETERS**	
	• Position of drain • Function of the drain	• Date & time of insertion of drain / catheter • Wound dressing
4	**SKIN PREPARATION**	
	• Umbilical preparation • General Cleanliness • Skin condition	• Bath • Site marking for preoperative patients • Shaving / Clipping if necessary
5	**ENVIRONMENT**	
	• Bed side rails up / Incubator door locked • Level of the bed • Bed breaks • Floor dry • Functioning of the call bell	• No clutter around the patient • Bedside locker is kept clean • Bathroom is clean & no washing basin inside • Machines should be clean • Functioning of the bed
6	**DOCUMENTATION / MEDICAL RECORDS**	
	DOCTORS ORDER: • Correct Patient Name • The physician order has noted & orders carried, date and time and nurse signature • Presence of physician's history and assessment **CONSENT FROMS:** • Signature and stamp of the physician and witness Complications written for high risk operations Date on the consent Explanation of the reason for a consent (diagnosis) and procedure to be done in English and Arabic.	**NURSES NOTES** • All pages with correct name • Nursing care plan updated • Nursing history and physical assessment checklist complete (Initial Assessment) • Signature of the admitting nurse • Date and time of admission • Pain scale complete • Fall risk assessment & Morse Scale score • Braden Score complete • Restraints • Intake& Output, Vital signs, Skin integrity and blood sugar monitoring chart • Correct patient Name Date / Time • Update data Nurses progress notes to check the following: • Last entry must have initials, Name & Job Number • Each entry must be with an initial & Job Number • Handwriting eligible
7	**SAFETY BRIEFING**	
	• Risk to patient (e.g. 2 patients with same name) • Near misses / adverse events • Adverse reactions • Post incidents learning / reviews / reflections • Staffing (nursing / medical) • Patients with mental health issues • Unauthorized persons / visitors	• Security • Equipment • Unfinished duties at shift change • Infection control • Changes in treatment (e.g. drug discontinuation) • Referrals • Changes in practice (e.g. change in lancets for blood glucose testing)